

RUN DATE:
RUN TIME:
BY:

UUID:1EE92799-E860-4C6B-B525-0C238D8B2E2F
TCGA-S5-AA26-01A-PR Redacted

PAGE:1



SPEC #:
STATUS:

Obtained:
Received:

Subm Dr:

CLINICAL HISTORY:

BLADDER CANCER

ICD-03
Carcinoma, papillary
urothelial 8130/3
Site: Bladder wall
C67.9
JW 2/25/14

SPECIMEN/PROCEDURE:

1. URETER - RIGHT DISTAL URETER
2. URETER - LEFT DISTAL URETER
3. URETHRA BIOPSY - DISTAL URETHRAL MARGIN
4. BLADDER, NOS
5. LYMPH NODE, ILIAC - LEFT COMMON
6. LYMPH NODE - LEFT OBTURATOR
7. LYMPH NODE, ILIAC - LEFT EXTERNAL
8. LYMPH NODE, ILIAC - RIGHT COMMON
9. LYMPH NODE - RIGHT OBTURATOR
10. LYMPH NODE, ILIAC - RIGHT EXTERNAL

IMPRESSION:

- 1) URETER, RIGHT DISTAL MARGIN, BIOPSY:
 - . Segment of ureter, negative for malignancy.
- 2) URETER, LEFT DISTAL MARGIN, BIOPSY:
 - . Segment of ureter, negative for malignancy.
- 3) URETHRA, DISTAL MARGIN, BIOPSY:
 - . Urethral tissue, negative for malignancy.
- 4) BLADDER AND PROSTATE, RADICAL CYSTOPROSTATECTOMY:
 - . BLADDER:
 - . Invasive papillary urothelial carcinoma, high-grade (WHO 2004/ISUP; see case summary.
 - . Carcinoma invades outer half of muscularis propria and invades perivesical tissue (microscopically).
 - . Lymphovascular and extensive perineural invasion are identified.
 - . Margins of resection negative for malignancy but close; carcinoma is present within lymphovascular space <1.0 mm from the left posterior perivesical soft tissue margin of resection.
 - . PROSTATE:
 - . Negative for urothelial malignancy.
 - . A few groups of atypical glands suspicious but not diagnostic of adenocarcinoma.
 - . Focal high grade prostatic intraepithelial neoplasia.
 - . Glandular and stromal hyperplasia with focal chronic inflammation and focal glandular atrophy.
 - . SEMINAL VESICLES, BILATERAL:
 - . Negative for malignancy.
- 5) LYMPH NODES, LEFT COMMON ILIAC, BIOPSY:

** CONTINUED ON NEXT PAGE **

IMPRESSION: (continued)

- . One lymph node, negative for malignancy (0/1).
- 6) LYMPH NODE, LEFT OBTURATOR, REGIONAL DISSECTION:
 - . Eleven lymph nodes, negative for malignancy (0/11).
- 7) LYMPH NODE, LEFT EXTERNAL ILIAC, BIOPSY:
 - . One benign lymph node (0/1).
- 8) LYMPH NODE, RIGHT COMMON ILIAC, REGIONAL DISSECTION:
 - . Two lymph nodes, negative for malignancy (0/2).
- 9) LYMPH NODES, RIGHT OBTURATOR, REGIONAL DISSECTION:
 - . Four lymph nodes, negative for malignancy (0/4).
- 10) LYMPH NODES, RIGHT EXTERNAL ILIAC, REGIONAL DISSECTION:
 - . Two lymph nodes, negative for malignancy (0/2).

* Pathologic TNM (AJCC 7th Edition): pT3a pN0

URINARY BLADDER: CYSTECTOMY, PARTIAL, TOTAL, OR RADICAL; ANTERIOR EXENTERATION CASE SUMMARY

SPECIMEN

Bladder
Prostate

PROCEDURE

Radical cystoprostatectomy

TUMOR SITE

Left lateral wall
Left posterior wall

TUMOR SIZE

Greatest dimension: 5.0 cm
Additional dimensions: 4.2 x 2.0

HISTOLOGIC TYPE

Urothelial (transitional cell) carcinoma

ASSOCIATED EPITHELIAL LESIONS

None identified

HISTOLOGIC GRADE

Urothelial carcinoma
High-grade

TUMOR CONFIGURATION

Papillary

MICROSCOPIC TUMOR EXTENSION

Tumor invades perivesical tissue
Microscopically

GROSS DESCRIPTION: (continued)

with the patient's name. The specimen consists of a 1.1 x 0.3 x 0.2 cm pale tan tissue fragment. The specimen is submitted entirely for frozen section. The frozen tissue is resubmitted in cassette 2F.

FROZEN SECTION DIAGNOSIS: Negative for tumor.

== Called to the OR at

by

3. Received fresh for frozen section labeled "distal urethral margin" per requisition and with the patient's name. The specimen consists of 3 fragments of pale tan tissue ranging in size from 0.4 x 0.2 x 0.2 cm to 1.3 x 0.4 x 0.3 cm the specimen is submitted entirely for frozen section. The frozen tissue is resubmitted in cassette 3F.

FROZEN SECTION DIAGNOSIS: Negative for tumor.

== Called to the OR at

by

4. Received fresh labeled with the patient's name, unit number and "bladder," is a radical cystectomy specimen containing bladder (7 x 6.5 x 3 cm) which has been pre-opened prior to arriving in the surgical pathology department. There is a brown-tan papillary tumor (4.2 x 5 x 2 cm) located at the left lateral and left posterior wall. The bladder has been opened in an irregular fashion which dissects the tumor mass in the left posterior wall. The tumor appears to invade the muscularis propria. There is a diverticulum in the right lateral wall which is approximately 2.5 cm deep. There is a 0.3 x 0.3 cm small hemorrhagic area in the trigone area adjacent to the prostatic urethra. The remainder of the bladder mucosa is unremarkable. The prostate consists of tan-white parenchyma, unremarkable in appearance. The right seminal vesicle (3.5 x 2 x 2 cm) and left seminal vesicle (3 x 1.5 x 1.5 cm) and bilateral vas deferens (0.3 cm in diameter each) are unremarkable. The perivesicle soft tissue was examined and no lymph nodes were identified.

Ink code:

Black - resection margin near tumor and left seminal vesicle;

yellow - left prostate;

green - right prostate and right seminal vesicle

Cassette 4A: Right lateral posterior bladder wall.

Cassette 4B: Left lateral posterior bladder wall.

Cassette 4C: Anterior bladder wall, mid level.

Cassette 4D-4E: Posterior wall tumor adjacent to possible ureter.

Cassette 4G-4H: Tumor adjacent sections, possible deepest invasion (left posterior wall).

Cassette 4J: Tumor, left posterior wall

Cassette 4K: Tumor, left posterior wall

Cassette 4L-4M: Tumor closest to left seminal vesicle and left posterior base prostate.

Cassette 4N: Diverticulum.

Cassette 4P: Right seminal vesicle.

Cassette 4Q: Left seminal vesicle.

Cassette 4R: Right anterior base prostate.

Cassette 4S: Right posterior base prostate.

Cassette 4T: Right anterior prostate.

Cassette 4U: Right posterior prostate

Cassette 4V: Left anterior and posterior prostate.

Cassette 4W: Hemorrhagic area near the trigone (bladder neck).

Cassette 4X-4AA: Right anterior, left anterior, left posterior, right posterior prostate.

Cassette 4AB-4AC: Right posterior and left posterior prostate.

Cassette 4AD-4AE: Left posterior prostate

Cassette 4AG: Left apex perpendicular sections.

Cassette 4AH: Right apex perpendicular sections.

IMPRESSION: (continued)

MARGINS

Margins uninvolved by invasive carcinoma/carcinoma in situ/noninvasive high-grade urothelial carcinoma

Distance of carcinoma from closest margin: carcinoma present within lymphovascular space <1.0 mm from the left posterior perivesical soft tissue margin (slide 4M)

LYMPH-VASCULAR INVASION

Present

PATHOLOGIC STAGING (pTNM)

PRIMARY TUMOR (pT)

pT3:: Tumor invades perivesical tissue

pT3a: Microscopically

REGIONAL LYMPH NODES (pN)

pN0: No lymph node metastasis

DISTANT METASTASIS

Not applicable

ADDITIONAL PATHOLOGIC FINDINGS

Diverticulum, urinary bladder

Organizing thrombi, pericystic and periprostatic tissue

Dictated by:

Entered:

COMMENT:

Permanent sections support the frozen section diagnoses (1-3).

Entered:

SPECIAL STAINS/PROCEDURES:

FROZEN SECTION SUMMARY:

One initial frozen section (1F).

One initial frozen section (2F).

One initial frozen section (3F).

Dictated by:

GROSS DESCRIPTION:

1. Received fresh for frozen section labeled "right distal ureter" per requisition and with the patient's name. The specimen consists of a 0.4 cm in diameter by 0.2 cm in length portion of ureter. The specimen is submitted entirely for frozen section. The frozen tissue is resubmitted in cassette 1F.

FROZEN SECTION DIAGNOSIS: Ureter margin negative for tumor.

== Called to the OR at

by Doctor

2. Received fresh for frozen section labeled "left distal ureter" per requisition and

** CONTINUED ON NEXT PAGE **

GROSS DESCRIPTION: (continued)

Cassette 4AJ-4AK: tumor, left posterior wall (adjacent sections, yellow ink used to indicate transection)

Cassette 4AL-4AM: tumor, left posterior wall (adjacent sections)

Cassette 4AN-4AP: tumor, left posterior wall (adjacent sections)

Cassette 4AQ-4AR: tumor, left posterior wall (adjacent sections, yellow ink used to indicate transection)

5. Received in formalin labeled with the patient's name, is an aggregate of irregular portions of yellow-tan lobulated adipose tissue, 2.2 x 1.5 x 0.7 cm. The specimen is dissected for lymph nodes, there is one tan ovoid lymph node identified, 1.3 x 0.7 x 0.4 cm. All lymph nodes identified are submitted as follows:

Cassette 5A: 1 lymph node bivalved

6. Received in formalin labeled with the patient's name, is an aggregate of irregular portions of yellow-tan lobulated adipose tissue, 5 x 5 x 1.3 cm. The specimen is dissected for lymph nodes, there are 9 tan ovoid lymph node identified, ranging from 0.7 x 0.4 x 0.2 cm to 2.7 x 0.9 x 0.7 cm. All lymph nodes identified are submitted as follows:

Cassette 6A: 4 lymph nodes

Cassette 6B: One lymph node bivalved

Cassette 6C: One lymph node bivalved

Cassette 6D: One lymph node bivalved

Cassette 6E: One lymph node bivalved

Cassette 6G: One lymph node bivalved

7. Received in formalin labeled with the patient's name, is an irregular portion of yellow-tan lobulated adipose tissue, 2.7 x 1.5 x 0.9 cm. The specimen is dissected for lymph nodes, there is one tan ovoid lymph node identified, 2.2 x 0.9 x 0.6 cm. All lymph nodes identified are submitted as follows:

Cassette 7A: One lymph node bivalved

8. Received in formalin labeled with the patient's name, is an aggregate of irregular portions of yellow-tan lobulated adipose tissue, 3.5 x 3 x 1.2 cm. The specimen is dissected for lymph nodes, there are 2 tan ovoid lymph nodes identified, 1 x 0.7 x 0.3 cm and 1.1 x 0.6 x 0.3 cm. All lymph nodes identified are submitted as follows:

Cassette 8A: 2 lymph nodes

9. Received in formalin labeled with the patient's name, is an irregular portion of yellow-tan lobulated adipose tissue, 7 x 2 x 1 cm. The specimen is dissected for lymph nodes, there are 4 tan ovoid lymph nodes identified, ranging from 0.3 x 0.2 x 0.2 cm to 3 x 1.1 x 0.6 cm. All lymph nodes identified are submitted as follows:

Cassette 9A: 3 lymph nodes

Cassette 9B: One lymph node bivalved

Cassette 9C: One lymph node bivalved

10. Received in formalin labeled with the patient's name, is an aggregate of irregular portions of yellow-tan lobulated adipose tissue, 5.5 x 3 x 0.8 cm. The specimen is dissected for lymph nodes, there 2 tan ovoid lymph nodes identified, 0.9 x 0.8 x 0.3 cm and 1.5 x 0.8 x 0.3 cm. All lymph nodes identified are submitted as follows:

Cassette 10A: One lymph node

GROSS DESCRIPTION: (continued)

Cassette 10B: One lymph node

Dictated by:
Entered:

COPIES TO:

CPT Codes:

FS INITIAL/88331/3, LYMPH NODE, REGIONAL RESECT/88307/4, PROSTATE RADICAL RESECTION/88309,
URETER BIOPSY-88305/2, URETHRA BIOPSY-88305, BLADDER CYSTECTOMY/88309,
LYMPH NODE BIOPSY/88305/2

ICD9 Codes:

189.9
Resident Physician:

I have personally reviewed the material
(specimen/slide) and approve this final report.

Electronically Signed by: _____

** END OF REPORT **

Criteria	hw 12/20/13	Yes	No
Diagnosis Discrepancy			✓
Primary/Tumor Site Discrepancy			✓
HIPAA Discrepancy			✓
Prior Malignancy History			✓
Dual/Synchronous Primary Noted			✓
Case is (circle):	QUALIFIED	DISQUALIFIED	
Reviewer Initials	BT	Date Reviewed	12/20/13